

We print the number. Everyone else blurs it.

Rate intelligence for the brokers and consultants who advise self-funded employer groups. What health plans have actually agreed to pay, in every market, with the sample size and the date on the face of it.

THE PROBLEM

Your client funds every claim and has never been shown a price.

A self-funded employer pays medical claims out of its own bank account. Seven figures a year is routine. They find out what care cost only after the money is gone.

Nobody hid it. Since 2022, federal law has required every health plan in America to publish exactly what it pays every doctor and hospital. The prices have been public for four years.

They are also unreadable. Blue Shield of California alone publishes **1.9 terabytes** across 1,249 files. One carrier, one state. Reading it takes engineering, not curiosity, so nobody on the buying side has read a line.

That is the entire opportunity. The most valuable price list in American healthcare is sitting in the open, and the people paying the bills cannot open it.

What changed for the broker

- Broker compensation is now itemized in writing in front of the client. Every fee needs something to point at.
- Plan sponsors attest annually that nothing blocks their access to price data. Almost none of them have ever seen it.
- Employers self-fund earlier and smaller every year. The one paying claims is now the one asking questions.

THE NUMBER

Same procedure. Same city. Six hundred percent apart.

A diagnostic knee arthroscopy in Sacramento, priced by where it happens. Federal allowed amounts, current quarter, physician fee included in every column.

SETTING	WHAT THE PLAN PAYS
Physician office	\$541.39
Ambulatory surgery center	\$2,186.26
Hospital outpatient department	\$3,884.26

+617%

Office to hospital outpatient. The same procedure, on the same patient, in the same city. The plan pays whichever building the claim landed in.

And the commercial market behind it

SACRAMENTO	MIDDLE	TOP	FILINGS
Brain MRI	\$591.97	\$1,535.49	22,232
Knee MRI	\$387.53	\$1,190.15	20,618

Every figure carries its sample size and the month it was filed. When the sample is too thin to trust, we refuse to print it and say so on the page.

WHAT WE HOLD

The corpus.

COMMERCIAL CONTRACTED RATES

What named health plans have agreed to pay, at metro grain, across every California market and building nationally.

OUT-OF-NETWORK ARBITRATION

5.33 million federal No Surprises Act dispute lines.
What arbitrators actually awarded against the plan's own benchmark.

SELF-FUNDED EMPLOYERS

Which employers carriers have publicly filed as self-funded, keyed to federal employer identifiers.

FEDERAL ALLOWED AMOUNTS

Physician fee schedule, hospital outpatient and surgery center files, current quarter, by real Medicare locality rather than a state average.

GROUP AND PROVIDER IDENTITY

Every practising clinician, their group, their tax ID and their market, so a rate attaches to a real organisation.

PAYER LITIGATION

15,813 federal dockets naming carriers as defendants, by district and nature of suit.

California, measured

CUT	SCALE
Rate observations behind the California layer	8.55M
California metros served at metro grain	33
Shoppable basket coverage across those metros	95.5%
California arbitration lines	123,723
Median award against the plan's own benchmark	2.57×

What a broker actually does with it.

Market rate LIVE

Any metro, any procedure. The distribution of what plans pay, with the number of filings and the month behind every figure.

Site of care LIVE

The same procedure priced in an office, a surgery center and a hospital outpatient department, with the facility fee joined to the physician fee. Most tools drop the physician fee, which makes hospitals look cheaper than offices. That is backwards, and it is wrong in front of a client.

Out-of-network exposure LIVE

What federal arbitration actually awards when a claim goes out of network. Self-funded plans budget these at the benchmark. The awards say otherwise.

Network comparison LIVE

The same basket of procedures, priced across the networks a client can actually buy, in dollars.

The opportunity book LIVE

A working database of employers in a territory, filterable by market, industry and size. Identity from public record, never invented.

Ask anything LIVE

A question in plain English returns a chart, a table or a finished document. The engine returns the figures; the language model may only narrate numbers it was handed.

The deliverable LIVE

A dated, sourced exhibit carrying the broker's own agency name, generated in seconds, forwardable to a client who has no account and never will.

The same law covers drugs. We read that too.

Pharmacy is roughly a quarter of plan spend, and specialty drugs are half of that, driven by a handful of members. It is the largest line item nobody can audit.

The drug layer LIVE

SOURCE	WHAT IT SETTLES
Acquisition cost	What the pharmacy actually paid for the drug. Federal, weekly.
Administered drug pricing	What infusions and injectables actually cost. Quarterly.
Patent and biosimilar calendar	When a client's most expensive drug is about to get cheap.
Discount-eligible facilities	Which hospitals buy drugs at a deep discount and bill full price.

The same drug. The same dose. The same patient. The price depends on the building. That is the site-of-care story applied to the biggest cost nobody examines.

The benefit manager LIVE

Three companies control roughly eighty percent of prescriptions, and all three are owned by the largest insurers. They set the price, pay their own pharmacies, and report on themselves. The spread between what a plan is charged and what the pharmacy was paid is legal, invisible, and now the subject of lawsuits against plan sponsors personally.

The employer is the fiduciary. The CFO signs. Nobody has ever handed them the number to check.

WHY IT WINS

Three things nobody else will do.

1. We print the dollar

The category shows a name and a rating and blurs the price until you book a demo. We show the price and withhold the name until you are a customer. That is not a marketing choice. It is the security boundary the product already enforces, which is why it stays true.

2. We publish what we refused

When a market is too thin to speak for, the page says so, names the threshold and prints how many cells were discarded. A vendor whose headline is volume cannot afford to admit what it does not know. It is the most credible thing on the screen and it cannot be copied without a confession.

3. We built it for the agency nobody serves

Every comparable platform is demo-gated, enterprise-priced and sold to health systems and national carriers. **129,645 of the 135,100 insurance agency establishments in America have fewer than twenty people.** None of them can buy any of it. They can buy this in ninety seconds on a phone.

PLAN	PRICE
Free — the market view, permanently, no account	\$0
Broker Pro — monthly	\$149
Broker Pro — annual	\$1,490
Agency — annual, flat, whole firm	\$4,900

What to say in the room.

Everyone in this category will either blur the number until you book a demo, or show you a number without telling you what it is made of. We print the number, the sample size, the date, and the rows we refused to print.

The broker's argument, in one line

Every other broker in that meeting says they will shop the market and negotiate hard. None of them can prove anything. You put a dated page on the table showing what the plans actually pay for the procedures these employees actually use, in this city. It is the only thing in the room that is not an opinion.

What it changes

- **It wins finals meetings.** Evidence beats assertion, and no incumbent can answer it.
- **It answers the fee question.** Compensation is disclosed now. This is what you point at.
- **It compounds.** Every exhibit carries the agency name and sits in a client's plan file, where the next broker sees it.

Live today: market rate, site of care, out-of-network arbitration, network comparison, the opportunity book, natural-language analysis, and generated deliverables. Self-serve accounts and billing are live and taking payment. **Next:** the pharmacy and benefit-manager layers.

Ask them where the dollars went.

Every plan sponsor in America has a pharmacy contract nobody has ever audited. Not because they are careless. Because the contract was written so that auditing it is not possible.

Two words that mean nothing

"Pass-through." **"Transparency."** Neither has a standard definition, and both appear in almost every pharmacy contract sold today. A plan can be promised one hundred percent pass-through of rebates, receive a fraction of what the manufacturer paid, and the contract will have been honoured exactly as written.

How that is possible

Manufacturer money arrives under many names. Rebates. Administrative fees. Data fees. Purchasing-group fees. Service allowances. **A contract that promises to pass through every rebate says nothing at all about money that was never called a rebate.**

The definition of one word in one clause decides where the money lands. That clause was not written by the employer.

The manufacturer paid X.
Your plan received Y.
Where did the difference go?

Nobody asks it, because until now nobody could answer it. The list price of a drug is public. What a pharmacy actually paid for it is public. What plans report receiving back is public. **Those three numbers have never been put in the same row.**

Three public numbers, one row.

WE SHOW	WHICH ANSWERS
List price against real acquisition cost, per drug	The size of the pool the money came out of
What the plan was charged against what the pharmacy was paid	The spread, per prescription
Reported rebates against the manufacturer discount pool	How much of it actually arrived
Preferred formulary drugs ranked by list price	Whether the plan is steered to the costliest option
Infusion pricing by site of care	What the building is adding to the same drug

A higher list price can mean a bigger rebate for the manager and a bigger bill for the plan. Both facts live in the same drug.

Why this lands hardest

The employer is the fiduciary. Employees have begun suing their own employers for overpaying on drugs, and **"our manager told us it was pass-through" is not a defence.** The CFO signs. Nobody has ever handed them the number to check.

What the broker walks in with

Not a promise to renegotiate. A page showing what the drug cost, what the plan paid, and what is unaccounted for.

Every competitor sells the employer a better contract. We show them the one they already signed.